

Panic Disorder: Patient Information Leaflet

Recurrent, unexpected panic attacks combined with persistent worry about future attacks or their consequences, often leading to avoidance behavior.

What is Panic Disorder?

Panic Disorder is characterized not simply by having panic attacks — which can occur in many anxiety and mood conditions — but by the pattern of recurrent, unexpected attacks combined with ongoing worry about having another one, or a significant change in behavior to avoid triggering an attack. This anticipatory anxiety and behavioral avoidance often become more disabling than the attacks themselves, and without treatment can narrow a person's life considerably as they avoid more and more situations.

Common symptoms

- Sudden surge of intense fear or discomfort peaking within minutes
- Palpitations, pounding heart, or accelerated heart rate
- Sweating, trembling, or shaking
- Shortness of breath or a smothering sensation
- Feelings of choking
- Chest pain or discomfort
- Nausea or abdominal distress
- Dizziness, light-headedness, or feeling faint

What causes it

Panic Disorder arises from a combination of genetic vulnerability and dysregulation of the brain's fear-alarm system, which misfires in the absence of real danger. Many first

episodes follow a period of significant stress, a major loss, a physical illness, or, in

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breathlessness) which can trigger a self-reinforcing cycle where fear of the sensations produces more of the sensations.

Treatment options

Cognitive Behavioral Therapy — particularly with an interoceptive exposure component (deliberately and safely provoking feared bodily sensations to break the fear-of-fear cycle) — is highly effective and considered first-line. Medication, usually an SSRI or SNRI, is commonly used alongside therapy or when therapy access is limited, especially for moderate-to-severe presentations.

Self-help tips

- Reduce or eliminate caffeine and stimulant intake
- Practice slow, diaphragmatic breathing regularly, not only during attacks
- Engage in regular aerobic exercise, which reduces overall anxiety sensitivity
- Avoid using alcohol to 'calm down,' which worsens the disorder over time
- Maintain a regular sleep schedule
- Gradually re-enter avoided situations rather than escalating avoidance

Prognosis

Panic Disorder responds very well to treatment, with most people achieving substantial reduction in attack frequency and severity within a few months. Left untreated, it frequently leads to agoraphobia and significant lifestyle restriction. Relapse can occur during stressful periods, but people who have learned CBT skills typically manage recurrences more effectively.

When to seek urgent help

Seek immediate medical attention for a first-ever panic attack, new or unexplained chest pain, or if you cannot distinguish panic symptoms from a possible cardiac or other medical emergency. Seek urgent psychiatric review if

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avoidance has progressed to the point of being unable to leave home, or if panic is accompanied by thoughts of self-harm.

This material is for patient and family education only and does not replace individualized medical advice. For a personal diagnosis or treatment plan, consult Dr. Kushal Kharel or a qualified mental health professional.

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